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DEPARTMENT OF HEALTH AND HUMAN SERVICES
Centers for Medicare & Medicaid Services
File code [CMS-0042-NC]
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Request for Information; Health Technology Ecosystem
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Patient.com Summary Response

> "The natural effort of every individual to better his own condition…is so powerful a principle, that it is alone, and without any assistance, not only capable of carrying on the society to wealth and prosperity, but of surmounting a hundred impertinent obstructions with which the folly of human laws too often encumbers its operations."
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> -Adam Smith, *An Inquiry into the Nature and Causes of The Wealth of Nations*

Patient.com appreciates the opportunity to respond to the Centers for Medicare & Medicaid Services (CMS) Request for Information (RFI) CMS-0042-NC regarding digital health products for Medicare beneficiaries as well as the state of data interoperability and broader health technology infrastructure. As a digital application and company dedicated to empowering patients through healthcare administration support and accessible and actionable health information, we advocate for policies that enhance consumer understanding and engagement in their healthcare choices.

About Patient.com and why we believe that patient empowerment must start with consent—and empathy

At Patient.com, we believe the key to solving healthcare's inefficiencies isn't just technology—it's empathy. And that begins by naming what so many Americans experience: **Patient Flustration™**. It's the compound feeling of being both flustered and frustrated by a system that claims to be digital, yet still buries people in paperwork, portals, passwords, and phone trees.

This emotional friction leads to real-world consequences—collection issues, delayed communication, missed opportunities for efficiency, and unnecessary stress for patients and caregivers alike. It also places avoidable strain on the entire healthcare ecosystem. Patient.com is on a mission to end Patient Flustration by putting patients back in control—because when you empower the patient, you transform the system.

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According to Experian Health's *2025 State of Patient Access*[i] survey, more than **1 in 5 patients (22%)** report delays in care due to **insurance verification issues**, and **20%** encounter errors in medical records or billing *before* seeing a doctor. Over **43%** say they are likely to postpone care without an accurate estimate, and **95%** struggle to pay for healthcare at least occasionally. These aren't just administrative issues—they represent a human crisis that deepens health disparities.

Patient.com was purpose-built to relieve this burden. At the core of our platform are **Patient.com Consent to Care™** and **Consent to Share™**—secure, consent-based identity and data exchange protocols that allow patients to safely activate support from their care circle. Whether it's a professional navigator, a family member, or a provider, anyone the patient trusts can be authorized to help—without compromising privacy, delaying care, or adding friction.

Our infrastructure adheres to **NIST IAL2 and AAL2**[ii] standards for identity assurance and authentication. That means patients can share claims, benefits, and clinical data confidently and securely across systems. Much like the **FACT Act**[iii] enabled consumer access to credit scores, and the **21st Century Cures Act**[iv] mandated access to longitudinal health records, we believe **radical usability—not just radical transparency—is the next frontier**. And it must be built on **verifiable, auditable consent**.

Patient.com transforms that promise into practice—bridging trust, technology, and care.

We wholeheartedly agree with the words in the February 25th Executive Order[v], "…Empowering Patients with Clear, Accurate, and Actionable Healthcare Pricing Information" and "…to put patients first and ensure they have the information they need to make well-informed healthcare decisions." But, as this Order and others attest, not enough has been done to "[fulfill] the promise of radical transparency." Providers continue to have trouble accessing important pieces of their patients' clinical history and patients give up on digital solutions and are often left with notebooks of paper to keep track of disparate sources of their own medical encounters.

Similar to the way the Fair and Accurate Credit Transactions Act (FACT Act) of 2003 legislated consumer access to their credit report and the ability to correct inaccuracies, Section 4006 of the 21st Century Cures Act enacted "patients access to their electronic health information in a single, longitudinal format that is easy to understand, secure, and may be updated automatically." In 45 CFR § 164.524 the Healthcare Insurance Portability and Accountability Act (HIPAA)[vi] described the individual's right of access to their protected health information. This request must be granted by the covered entity in "…a timely manner…" "…in the form and format requested…" with few exceptions. While much data has been shared through the years

since HIPAA with requesting individuals, for anyone who has seen the average Continuity of Care Document, (CCD) the data can be poorly presented, duplicative, excessive and yet still have gaps and errors where data was not entered correctly, did not fit the format of transmission or the document itself.

In addition to poor presentation of data, regulatory, contractual and operating procedure requirements have been set with the intention of securing confidentiality and integrity, but not necessarily accessibility (CIA triad[vii]). Multiple steps are required that should not act as barriers: ID verification and authentication, access to all data related to the individual which includes claims, remittance advice, eligibility and benefit data and any type of clinical data whether data elements, reports or imaging. Specific areas where barriers should be lowered are within TEFCA contract terms, QHIN requirements, Electronic Data Interchange (EDI) ASC X12 Version 5010 and the Medicare HIPAA Eligibility Transaction System (HETS) constraint to provider requests.

The 21st Century Cures Act also states that "the National Coordinator and the Office for Civil Rights of the Department of Health and Human Services shall jointly promote patient access to health information in a manner that would ensure that such information is available in a form convenient for the patient, in a reasonable manner, without burdening the health care provider involved." In addition, "The Secretary, in consultation with the National Coordinator, shall promote policies that ensure that a patient's electronic health information is accessible to that patient and the patient's designees, in a manner that facilitates communication with the patient's health care providers and other individuals, including researchers, consistent with such patient's consent."

To achieve "the promise of radical transparency" **and usability** in healthcare data and ensure individuals "have the information they need to make well-informed healthcare decisions." We propose the following opportunities:

1. Resolve information blocking.
    a. Allow digital health products to use the "High Confidence" Identity Assurance Level 2 (IAL2) and Authenticator Assurance Level 2 (AAL2) (NIST SP 800-63-3) processes to access Clearinghouse, Blue Button 2.0 and QHIN clinical data in multiple ways. When patient consent is informed, stored and auditable, the patient should be able to access their healthcare data upon request and without extra blocking steps.
        i. A BAA should not be required with a Covered Entity as long as the digital health product has auditable patient consent and complies with IAL2 and AAL2 processes.
        ii. Digital Health Products should be able to use IAL2 and AAL2 service which

is developed and managed internally or from any suitable vendor and not only the six certified Credential Service Providers[viii].

iii. Instead of requiring individual login and password credentials, as with Blue Button 2.0 and other Payer Patient Access APIs, allow bulk access to the data using demographics gathered from the IAL2 process.

b. Reconsider the administrative necessity of the Common Agreement for Trusted Exchange Framework. A contract in this scenario may be limiting possible networks, participants and sub-participants from joining. Decreased management of this contract process would lead to cost reduction. Multiple Rules, Requirements and Standard Operating Procedures are well established to maintain a Trusted Exchange Framework.

c. Encourage and incentivize participation of Covered Entities in the Trusted Exchange Framework Common Agreement (TEFCA) Qualified Health Information Network (QHIN).

d. Penalize TEFCA QHIN participants and subparticipants who do not return data for the currently required exchange purposes of Treatment (T-TREAT) and Individual Access Services (T-IAS). Various disincentives are currently used by CMS, ASTP and OIG which are a progression from probation and then exclusion from the QHIN, or monetary penalties such as fines or reduction of Medicare payments as in the Hospital Readmissions Reduction Program. Unfortunately, the impacts of these programs have not yet had the desired effect and need advancement.

    i. The Recognized Coordinating Entity (RCE) manages the QHIN directory. Access to this listing remains limited to RCE and QHINs but should be made public as soon as possible and this could help pressure non-responding participants.

e. Create a modified version of TEFCA for Individual Access Services with terms that recognize the generation of queries with a patient's consent, invoking FTC Act[ix] and the Health Breach Notification Rule[x] obligations instead of HIPAA.

f. Ensure all data queried is returned without geofencing: limiting data returned within an arbitrary geographic region thought to be the most likely area within which a patient received clinical services.

g. Ensure the availability of complete price information in a standard data format for clinical products and services to patients prior to encounters and order fulfillment that is easy to access and comprehend. As per the Executive Order 14221, "The Secretary of the Treasury, the Secretary of Labor, and the Secretary of Health and Human Services shall take all necessary and appropriate action to rapidly implement and enforce the healthcare price transparency regulations".

Similar disincentives should be invoked as mentioned above: progression from probation and then exclusion from Medicare, or monetary penalties such as fines or reduction of Medicare payments as in the Hospital Readmissions Reduction Program.

2. Ensure the "Patients over Paperwork[xi]" commitment to "cut the red tape" and "…allow providers to concentrate on their primary mission: improving health outcomes".
    a. Allow digital health products to facilitate patients' access to ASC X12 Version 5010 transaction data through HETS and other clearinghouses without the necessity of a provider's credentials within the request. The current X12 transaction set requests are limited to providers. The existing EDI system should be accessible by digital applications on behalf of patients. This small change would unlock significant value for patients without technical changes for HETS or clearinghouses. Examples of request types that should be available to patients include claims or encounters (837), eligibility and benefit information (270) and referrals and authorizations for services (278).
    b. Remove the HETS provider affidavit requirement for ASC X12 Version 5010 transactions so that patients may consent to allow digital health products to leverage the HETS system for benefits, claims, and other data accessed by providers.
    c. Allow bulk ASC X12 Version 5010 transactions requested by digital products on behalf of patients without requiring provider credentials. For efficiency, digital health products should be able to obtain massive amounts of data on behalf of patients as long as identity is verified at IAL2 and consent informed, stored and auditable.
3. Advance Standards more quickly.
    a. Advance the other exchange purposes (XPs) from the TEFCA Standard Operating procedure V.3 August 6, 2024[xii] quickly. If the planned exchange purposes for Payment, Healthcare operations, Care Coordination, HEDIS and Quality Measure reporting were advanced to required, there would be a greater incentive and focus on all the exchange purposes being successfully activated.
    b. Regarding the United States Core Data for Interoperability (USCDI)[xiii]:
        i. Review the Standards Version Advancement Process (SVAP)[xiv] for any concerns and if none, promote the United States Core Data for Interoperability (USCDI) to the current approved version, 5.
        ii. Reinstate the Health Information Technology Advisory Board to continue

advancing USCDI.

iii. Review and approve new versions of USCDI faster, at least quarterly.

iv. Incorporate more data classes and elements in USCDI versions.

c. Incentivize QHIN and other health information networks for end-to-end FHIR exchange for quicker implementation. Suggest HHS create a pool of funds from for investment in QHIN participation, like what was appropriated during the Obama administrations.

d. Maintain the January 1, 2027 requirement for the CMS Interoperability and Prior Authorization Rule (CMS-0057-F)[xv]

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[v] Executive Order 14221. February 25, 2025: Making America Healthy Again by Empowering Patients with Clear, Accurate, and Actionable Healthcare Pricing Information. https://www.federalregister.gov/documents/2025/02/28/2025-03440/making-america-healthy-again-by-empowering-patients-with-clear-accurate-and-actionable-healthcare

[vi]Access of Individuals to Protected Health Information. Code of Federal Regulations, Title 45, Subtitle A, Subchapter C, Part 164, Subpart E, § 164.524. https://www.ecfr.gov/current/title-45/subtitle-A/subchapter-C/part-164/subpart-E/section-164.524

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[xiii] HealthIT.gov Interoperability Standards Platform, United States Core Data for Interoperability (USCDI). https://www.healthit.gov/isp/united-states-core-data-interoperability-uscdi

[xiv] Assistant Secretary for Technology Policy, Standards Version Advancement Process (SVAP). https://www.healthit.gov/topic/standards-version-advancement-process-svap

[xv] Centers for Medicare & Medicaid Services, Medicare and Medicaid Programs; Patient Protection and Affordable Care Act; Advancing Interoperability and Improving Prior Authorization Processes for Medicare Advantage Organizations, Medicaid Managed Care Plans, State Medicaid Agencies, Children's Health Insurance Program (CHIP) Agencies and CHIP Managed Care Entities, Issuers of Qualified Health Plans on the Federally-Facilitated Exchanges, Merit-Based Incentive Payment System (MIPS) Eligible Clinicians, and Eligible Hospitals and Critical Access Hospitals in the Medicare Promoting Interoperability Program 2/8/2024. https://www.federalregister.gov/documents/2024/02/08/2024-00895/medicare-and-medicaid-programs-patient-protection-and-affordable-care-act-advancing-interoperability